

[SYNTHETIC SAMPLE] Fictional record generated for the DELPHi prototype demo. Names, institutions, and statements are invented; the layout imitates a generic field-medical format. Not a real document and not derived from any real record.

[CONFERENCE REFERENCE] The meeting named above is a real, publicly announced scientific congress and is used here only as the setting of this fictional record. No session content, abstract, poster, or presented data from that meeting is reproduced or paraphrased; every person and conversation below is invented.

Congress Attendance Report — American Epilepsy Society Annual Meeting (AES) 2021

Meeting period: December 2021 | Author: Ingrid Halvorsen

Scope: unsolicited conversations held on-site. Session content and presented data are deliberately not reproduced here.

Conversation — Mairead Eversholt, MD (Granite Bay Epilepsy Center)

Approached me rather than the other way around, which is worth noting. Benefit verification is the step that most often delays a start here. The phone triage line is being piloted this quarter, and that took up the first part of the conversation. The composition of the older half of the panel has shifted over the last year or two.

Conversation — Nadine Sandoval-Reyes, MD (Windrow Neuroscience Center)

A short exchange over the coffee queue. Titration scheduling is handled by nursing staff, which the practice finds workable. Described how long-standing patients are triaged now compared with before.

Conversation — Odalys Galbraith, MD (Bluestem Comprehensive Epilepsy Clinic)

Introduced by a colleague from the same region and talked for a few minutes. This practice is still building familiarity with the product and had no clinical points to raise. The composition of the older half of the panel has shifted over the last year or two.

Conversation — Xiomara Barrowman, MD (Beacon Hill Neurology Associates)

Approached me rather than the other way around, which is worth noting. The new charting templates changed again last month, and that took up the first part of the conversation. Mostly small talk about the program and the venue.

Conversation — Delphine Kirkpatrick, MD (Copperfield Neurology Institute)

Introduced by a colleague from the same region and talked for a few minutes. Monitoring intervals during titration are set by clinic routine here rather than by protocol. Mostly small talk about the program and the venue. Described how the surgical workup list is triaged now compared with before.

Conversation — Walter Brandt, MD (Flint Hills Neuroscience)

We spoke standing near the exhibit aisles, briefly. Referenced a review that circulated on the service in passing. Mentioned a session summary from a colleague but did not raise any specific finding. Noted that the appeals process has improved somewhat and that this is not specific to any one product.

Conversation — Felicity Bellweather, MD (Sandhill Neuroscience Center)

The conversation happened while waiting for a session room to clear. Talked through the older half of the panel in general terms, without any specific case. Nothing that needed follow-up this time. Asked about safety and confirmed nothing to report. Asked whether I had seen a case series someone forwarded; I had not, and offered nothing in return.

Conversation — Eloise Eversholt, MD (Quarry Hill Epilepsy Center)

Stopped to talk on the way out of a session block, with no agenda on either side. Coverage paperwork is handled by one person here, which came up as a general frustration. Onboarding one of the nurse practitioners is the current preoccupation here.

Conversation — Lachlan Dunmore, MD (Sandhill Neuroscience Center)

Approached me rather than the other way around, which is worth noting. Asked whether I would be at the meeting later this year. Talked through the refractory group in general terms, without any specific case.

Conversation — Valentina Calderwood, MD (Foxglove Health Neurology)

A hallway conversation during the afternoon break. Spent some time on the surgical workup list and how workup decisions get sequenced. Agreed to reconnect at the following quarter; no specific request.

Conversation — Thandiwe Barrowman, MD (Quarry Hill Epilepsy Center)

Approached me rather than the other way around, which is worth noting. Referral patterns into this clinic have changed, and newly referred patients is a bigger share than it was. The practice lost one of the nurse practitioners and has not backfilled, so the new-patient backlog is slowly recovering. Asked whether I had seen a guideline update discussed at journal club; I had not, and offered nothing in return.

Conversation — Malika Fairweather, MD (Highmoor Health Neurology)

Caught between sessions in the poster hall; the conversation was short and entirely unprompted. Coverage paperwork is handled by one person here, which came up as a general frustration. Asked whether I had seen a review that circulated on the service; I had not, and offered nothing in return.

Follow-ups have been logged in the interaction system.